INFORMATION TO KEEP WITH YOU

# My complete treatment handbook

## Dose-dense AC → weekly paclitaxel + weekly trastuzumab

Understand your treatment. Know what to expect. Keep your plan to hand.

**Displayed calendar**
**14 days**
Intervals differ by treatment phase

**Treatment stages**
**3**
Explained in the treatment reference

**Regimen source**
**NCCP 00433**
Version 6

Four AC cycles 14 days apart with growth-factor support, then 12 weekly paclitaxel doses with weekly trastuzumab. Continue trastuzumab to complete one year total; no concurrent AC / trastuzumab.

## Treatment stages

01 **Phase 1 · AC**
14-day interval

02 **Phase 2 · Paclitaxel with HER2 treatment**
7-day interval

03 **Review / HER2 continuation**
21-day interval

Your team confirms the active stage and dates. The full treatment guide explains each step.

## Inside your handbook


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**My name** _____

**Hospital / oncology team** _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

# Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

Dose-dense AC → weekly paclitaxel + weekly trastuzumab
National Cancer Control Programme (NCCP) 00433

## When to get help

**Emergency: call 112 / 999**

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

**Contact your oncology service immediately**

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Contact oncology promptly for fever, shivering, unusual bleeding, severe mouth or bowel symptoms, inability to drink, reduced urine, new breathlessness or a painful/leaking line. Severe chest pain, collapse or severe breathing difficulty needs emergency help.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

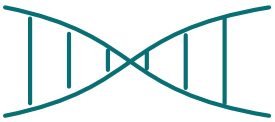
This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

## How the medicines work

Before surgery: treat the cancer and support the agreed surgical plan. After surgery: reduce recurrence risk. Confirm which source indication and sequence apply.

ANTHRACYCLINE CHEMOTHERAPY

### Doxorubicin



**Interferes with deoxyribonucleic acid (the cell's genetic material) (DNA) handling**

Doxorubicin interferes with enzymes and DNA processes needed for cell division. It can also affect heart muscle, so the team considers previous exposure and cardiac risk.

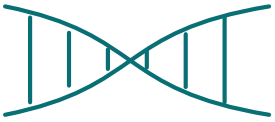
DNA handling disrupted → Cell damage → Division reduced

**What to watch for**

Report pain or burning at the infusion site immediately. New breathlessness or chest symptoms need prompt review. Red urine for a short time can be a drug-colour effect; pain, clots or persistent bleeding still need advice.

ALKYLATING CHEMOTHERAPY

## Cyclophosphamide



### Damages DNA needed for growth

Cyclophosphamide is activated in the body and damages DNA. This helps stop dividing cancer cells but can also affect blood-cell production and reproductive tissues.

Drug activated → DNA damage → Cell division affected

### What to watch for

Use the prescribed anti-sickness and hydration plan. Report fever, painful urination or blood in the urine.

TAXANE CHEMOTHERAPY

## Paclitaxel

Taxol (conventional paclitaxel)



### Locks the cell division machinery

Cells use tiny structures called microtubules to separate their chromosomes when dividing. Paclitaxel stabilises these structures so they cannot rearrange normally, interrupting cell division. It can also affect nerves and other healthy tissues.

Microtubules form → Structures remain locked → Division is interrupted

### What to watch for

Report tingling or numbness before the next weekly dose, especially if it affects walking or hand use. Tell the nurse immediately about flushing, chest or throat tightness, itching, dizziness or breathing changes during infusion.

HER2-TARGETED ANTIBODY

**Trastuzumab**



### Targets HER2 growth signals

Trastuzumab binds to the HER2 protein on cells and helps reduce HER2-driven growth and recruit immune activity. HER2 testing identifies the cancers for which this approach is used.

HER2 on cell surface → Antibody binds → Growth signalling reduced

**What to watch for**

Keep echo appointments throughout HER2 treatment. Report breathlessness, swelling, palpitations or a reaction; chemotherapy side effects are tracked separately.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

## Why this treatment?

Dose-dense AC is followed by weekly paclitaxel and weekly trastuzumab for HER2-positive high-risk early breast cancer. The different phases have different medicines, growth-factor requirements and supportive prescriptions.

### Treatment setting

HER2-positive neoadjuvant or adjuvant high-risk breast cancer

### Your exact schedule

Four AC cycles 14 days apart with growth-factor support, then 12 weekly paclitaxel doses with weekly trastuzumab. Continue trastuzumab to complete one year total; no concurrent AC / trastuzumab.

### Alternatives

Discuss suitable endocrine, targeted or chemotherapy options, expected benefit, practical demands and the option of supportive care according to your setting.

## Why this treatment may be chosen

### Show trial results and what the numbers mean

- Pembrolizumab + chemotherapy
- Placebo + chemotherapy

Your team can discuss expected benefit, alternatives and uncertainty for your own cancer.

CHAPTER 02

# Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

## The phases and how they connect

### Treatment stages

01 **Phase 1 · AC**
14-day interval

02 **Phase 2 · Paclitaxel with HER2 treatment**
7-day interval

03 **Review / HER2 continuation**
21-day interval

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

14-DAY INTERVAL

### Phase 1 · AC

**4 cycles**

Four AC cycles 14 days apart. Growth-factor support is part of this dose-dense schedule.

7-DAY INTERVAL

### Phase 2 · Paclitaxel with HER2 treatment

**12 weekly taxane doses**

Four AC cycles 14 days apart with growth-factor support, then 12 weekly paclitaxel doses with weekly trastuzumab. Continue trastuzumab to complete one year total; no concurrent AC / trastuzumab.

21-DAY INTERVAL



### Review / HER2 continuation

**Individualised after review**

Continue the source 21-day HER2 schedule after chemotherapy to complete one total year, unless clinical findings change treatment.

The active phase determines the medicines and support prescription. Heart monitoring continues through the change to trastuzumab alone.

BASELINE → TREATMENT → FOLLOW-UP

## When checks happen

Relative milestones, not equal time intervals. Your team books tests and reviews results; this map does not confirm that either has happened.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00433 v6 · official regimen · ESMO 2024 early breast cancer guideline · treatment and follow-up · NICE NG101 · bone health and follow-up · EMA Herceptin product information

THE WHOLE TREATMENT JOURNEY

# Dose-dense AC → weekly paclitaxel + weekly trastuzumab: treatment and checks

**Read from left to right.** Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

## Treatment, tests and reviews across the course

| Care / check | Before starting | First treatment block | During treatment | Change / completion | Follow-up |
|---|---|---|---|---|---|
| **Cancer medicines** NCCP 00433 | Confirm exact regimen | Dose-dense AC → weekly paclitaxel + weekly trastuzumab | Four AC cycles 14 days apart with growth-factor support, then 12 weekly paclitaxel doses with weekly trastuzumab. Continue trastuzumab to complete one year total; no concurrent AC / trastuzumab. | Review active medicines and purpose | Next treatment / follow-up plan |
| **Surgery / radiotherapy** NCCP 00433 | HER2-positive neoadjuvant or adjuvant high-risk breast cancer | Follow the multidisciplinary team (MDT) sequence | Radiotherapy timing needs agreement | Selected local treatment as indicated | Review recovery and ongoing effects |
| **Blood / kidney / liver** NCCP 00433 | full blood count (FBC), renal and liver profile | Confirm treatment-day results | Repeat on the source/local schedule | Review unresolved abnormalities | Individual clinical plan |
| **Heart / breathing** NCCP 00433 · SMPC | Baseline cardiac assessment | Reassess before trastuzumab | EMA: cardiac assessment every 3 months | Keep clock through phase change | Product-information / risk-based plan |
| **Cancer review** NCCP 00433 · EARLY | Confirm stage and setting | Agree review checkpoints | Review symptoms and response | Discuss continuation / next treatment | Breast follow-up |

| Care / check | Before starting | First treatment block | During treatment | Change / completion | Follow-up |
|---|---|---|---|---|---|
| **Other checks**<br>NCCP 00433 · BONE | Confirm HER2 and baseline eligibility | Review support medicines | Review toxicity before each dose | Endocrine / bone plan where relevant | Follow unresolved symptoms |

## What each check is for

1

### Baseline review

Before treatment

HER2-positive neoadjuvant or adjuvant high-risk breast cancer. Review lifetime anthracycline exposure and baseline cardiac risk before AC; reassess before starting trastuzumab. NCCP specifies cardiac tests as clinically indicated; EMA trastuzumab information specifies baseline and three-monthly cardiac assessment during treatment. Keep the monitoring clock through the change to trastuzumab alone; arrange earlier review for symptoms or falling function. EMA product information recommends cardiac checks every 6 months after treatment until 24 months; following anthracyclines in early disease, yearly up to 5 years from the last trastuzumab dose or longer if function falls. Your team individualises this plan.

**Ask** / **act:** Confirm source eligibility and results.

2

### Treatment-day check

Before each planned dose

Baseline blood tests and review before each chemotherapy administration according to the source and local plan. Once chemotherapy ends, testing follows trastuzumab and individual clinical needs.

**Ask** / **act:** Confirm active medicines and whether a dose is held.

3

### Support plan

At every phase or day change

Four AC cycles 14 days apart with growth-factor support, then 12 weekly paclitaxel doses with weekly trastuzumab. Continue trastuzumab to complete one year total; no concurrent AC / trastuzumab.

**Ask** / **act:** Choose the matching support script; do not merge phases.

4 **Response and follow-up**

Agreed review points

After early-breast treatment, breast imaging and clinical review follow the agreed plan. Routine computed tomography (CT)/positron emission tomography (PET) scans or blood tumour markers are not used to look for distant recurrence in a person without symptoms or clinical concerns.

**Ask** / **act:** Review symptoms, results and patient priorities together.

**At the next review, ask**

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

## Your practical treatment plan

STEP 1

### Before the first visit

Blood tests, medicines review and the consent discussion come first. Confirm baseline and planned ongoing cardiac assessment.

STEP 2

### Plan venous access

A cannula, peripherally inserted central catheter (PICC) or implanted port may be used. If a port is chosen, a trained team inserts it in a separate procedure with local anaesthetic and sometimes sedation; the team explains fasting, blood-thinner and wound instructions.

STEP 3

### Infusion day

Staff check the line, give prescribed premedicines and treatment, and observe for reactions. Tell them immediately about burning, pain, rash or breathlessness.

STEP 4

### Home and next review

This regimen does not routinely need a take-home infusion pump. Staff disconnect the infusion; a port is de-accessed when appropriate. Keep line-care and next-visit instructions.

### Port insertion and removal

The line team explains placement, dressing care and when normal activities can resume. A port is accessed with a special needle during treatment. Removal of the implanted device is a separate planned procedure when no longer needed; do not remove or adjust it yourself.

### PICC care and urgent line symptoms

A PICC usually needs regular dressing and flushing appointments even between treatments. Fever, redness, swelling, pain, leakage or damage requires prompt advice. Your hospital supplies the exact care instructions.

### Visit time and transport

First visits can take longer because of loading doses, premedicines and observation. Ask the unit about expected time, transport and any escort needed after sedation or line insertion.

### Phase-specific visit pattern

Four AC cycles 14 days apart with growth-factor support, then 12 weekly paclitaxel doses with weekly trastuzumab. Continue trastuzumab to complete one year total; no concurrent AC / trastuzumab. No take-home infusion pump is part of this schedule. Port de-accessing after a visit is different from removal of the implanted port.

## Checks across treatment

Your clinical team books tests, reviews results and explains any change. Contact them if an expected test or instruction is missing.

### Blood and kidney tests

Baseline blood tests and review before each chemotherapy administration according to the source and local plan. Once chemotherapy ends, testing follows trastuzumab and individual clinical needs.

### Heart and breathing

Review lifetime anthracycline exposure and baseline cardiac risk before AC; reassess before starting trastuzumab. NCCP specifies cardiac tests as clinically indicated; EMA trastuzumab information specifies baseline and three-monthly cardiac assessment during treatment. Keep the monitoring clock through the change to trastuzumab alone; arrange earlier review for symptoms or falling function. EMA product information recommends cardiac checks every 6 months after treatment until 24 months; following anthracyclines in early disease, yearly up to 5 years from the last trastuzumab dose or longer if function falls. Your team individualises this plan.

### Cancer imaging

After early-breast treatment, breast imaging and clinical review follow the agreed plan. Routine CT/PET scans or blood tumour markers are not used to look for distant recurrence in a person without symptoms or clinical concerns.

### Bone health

Bone-density testing follows endocrine treatment, ovarian suppression, menopause and other bone-risk factors. dual-energy X-ray absorptiometry (bone density) (DEXA) is not automatically required before every chemotherapy course.

## The wider treatment plan

For neoadjuvant treatment, agree response review and surgery planning; for adjuvant treatment, surgery has usually happened. Follow the agreed early-breast pathway. Trastuzumab may continue after chemotherapy and can overlap radiotherapy when approved by the treating team. Endocrine therapy depends on hormone receptors. Do not give trastuzumab concurrently with the AC phase.

### When treatment changes

Reconcile active cancer medicines, supportive medicines and remaining symptoms before every transition.

### Follow-up

Keep the agreed review appointments and report new symptoms between visits; this map does not detect missed tests.

### Hormone treatment and bone care

Hormone-receptor results and your treatment setting determine whether endocrine therapy is appropriate. It has its own monitoring plan.

CHAPTER 03

# Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

# AC · 14-day reference cycle

## Day 1 → Day 14: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

AC · 14-day reference cycle

| Treatment or check | Timing in this view |
|---|---|
| **Treatment · Day 1** |  **Day 1** intravenous (IV) |
| **Administration reaction** |  **Days 1–14** Report during or after any visit |
| **Nausea / appetite** |  **Days 1–5** May occur early or later |
| **Blood counts / infection** |  **Days 1–14** Risk varies; fever needs prompt advice |
| **Fatigue / mouth changes** |  **Days 1–14** Variable timing and severity |

**No dates to enter:** The chart explains the pattern. Your team books actual appointments and reviews results.

**Phase changes:** Supportive medicines and symptom expectations change when chemotherapy stops. HER2-only treatment does not automatically inherit chemotherapy premedicines.

**Treatment delays:** A delayed dose does not automatically delay an echo or other test that is already due.

**Why these bands stay broad:** These broad symptom bands are educational possibilities, not measured risk curves. Infusion days are the planned schedule; delays require a new clinical plan. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00433 v6 · official regimen ↗

**There are no guaranteed “safe days”.** Fever, shivers, sudden illness, severe breathing changes or chest pain need prompt assessment on any day.

# Phase 2 · Paclitaxel with HER2 treatment

## Day 1 → Day 7: a general pattern

Phase 2 · Paclitaxel with HER2 treatment

| Treatment or check | Timing in this view |
|---|---|
| **HER2 medicine · Day 1** |  **Day 1** IV |
| **Chemotherapy · Day 1** |  **Day 1** IV |
| **Administration reaction** |  **Days 1–7** Report during or after any visit |
| **Nausea / appetite** |  **Days 1–5** May occur early or later |
| **Blood counts / infection** |  **Days 1–7** Risk varies; fever needs prompt advice |
| **Fatigue / mouth changes** |  **Days 1–7** Variable timing and severity |
| **Nerves / skin / nails** |  **Days 1–7** Can build across doses |
| **Heart / breathing symptoms** |  **Days 1–7** Report on any day |

**No dates to enter:** The chart explains the pattern. Your team books actual appointments and reviews results.

**Phase changes:** Supportive medicines and symptom expectations change when chemotherapy stops. HER2-only treatment does not automatically inherit chemotherapy premedicines.

**Treatment delays:** A delayed dose does not automatically delay an echo or other test that is already due.

> **Why these bands stay broad:** These broad symptom bands are educational possibilities, not measured risk curves. Infusion days are the planned schedule; delays require a new clinical plan. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00433 v6 · official regimen ↗

# HER2-only continuation · source 21-day schedule

## Day 1 → Day 21: a general pattern

HER2-only continuation · source 21-day schedule

| Treatment or check | Timing in this view |
|---|---|
| **Treatment · Day 1** | 1 — 21<br>**Day 1**<br>IV |
| **Administration reaction** | 1 — 21<br>**Days 1–21**<br>Report during or after any visit |
| **Heart / breathing symptoms** | 1 — 21<br>**Days 1–21**<br>Report on any day |

**No dates to enter:** The chart explains the pattern. Your team books actual appointments and reviews results.

**Phase changes:** Supportive medicines and symptom expectations change when chemotherapy stops. HER2-only treatment does not automatically inherit chemotherapy premedicines.

**Treatment delays:** A delayed dose does not automatically delay an echo or other test that is already due.

> **Why these bands stay broad:** These broad symptom bands are educational possibilities, not measured risk curves. Infusion days are the planned schedule; delays require a new clinical plan. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00433 v6 · official regimen ↗

### Between-treatment notes

### Day 1–14: Your current reference cycle

Four AC cycles 14 days apart with growth-factor support, then 12 weekly paclitaxel doses with weekly trastuzumab. Continue trastuzumab to complete one year total; no concurrent AC / trastuzumab. Follow the team's appointment and medicine instructions; contact them about missed doses or new symptoms.

## Support around the medicines

**1** **Anti-sickness plan**

Follow the prevention and rescue instructions prescribed for this schedule. Call if vomiting prevents drinking.

**2** **Infection and bleeding**

Know the local urgent-contact number. Fever, shivering, sudden illness or unusual bleeding needs prompt advice even between visits.

**3**  **Medicine review**

Show the oncology pharmacist all prescribed, over-the-counter and herbal medicines. Ask before starting anything new.

**4** **Premedicines belong to the active phase**

Paclitaxel reaction medicines and AC anti-sickness medicines have different purposes. They are not automatically continued during trastuzumab-only treatment.

## Questions worth asking

### Who arranges the next tests?

Your treating service arranges and reviews tests. If instructions or appointments are missing, contact the team; the map does not book tests.

### What if a dose is missed or I feel unwell?

Contact the unit for a revised plan. Do not double tablets, shift treatment dates or assume that an omitted dose should be made up.

**Try explaining the plan back:** Which medicines are due on which days? What symptoms mean you should call today? Which tests come next and who arranges them?

### Doxorubicin

- Low counts, infection and mouth ulcers
- Nausea, hair loss and fatigue
- Heart damage, sometimes delayed
- Severe tissue injury if the infusion leaks
- Rare later blood cancers

### Cyclophosphamide

- Low counts and infection
- Nausea, vomiting and hair loss
- Bladder irritation / bleeding
- Fertility effects and menopause

### Paclitaxel

- Low blood counts and infection
- Numbness / tingling, sometimes lasting
- Hair loss, muscle or joint pain
- Infusion reaction
- Mouth, bowel, skin and nail effects

### Trastuzumab

- Heart pumping-function reduction / heart failure
- Infusion or allergic reaction
- Diarrhoea and fatigue
- Rare serious lung effects

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**Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

## AC · 14-day schedule

**Active medicines:** Doxorubicin + Cyclophosphamide

**Emetogenic risk:** High · AC

| Medicine | Dose / route | When |
|---|---|---|
| Aprepitant | 125 mg orally | Day 1 |
| Ondansetron | 16 mg orally | Day 1<br>Individual dose / QT / hepatic review |

| Medicine | Dose / route | When |
|---|---|---|
| Dexamethasone | 12 mg orally | Day 1<br>No routine Days 2–4 dexamethasone in this NCCP AC antiemetic row |
| Olanzapine | 5 mg orally | Days 1–4<br>Review sedation, falls, interactions and glucose |
| Aprepitant | 80 mg orally daily | Days 2–3 |
| Metoclopramide | 10 mg orally up to three times daily | Subsequent days: duration and use to be confirmed<br>Caution with olanzapine: additive extrapyramidal effects; adjust / avoid for individual contraindications |

### Additional phase-specific support

- This AC antiemetic script is replaced at the taxane transition.
- Growth-factor support is required for dose-dense AC. Specify the product, dose and days under local policy.
- Select the taxane support plan at the phase transition; do not continue the AC antiemetic course by default.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

## Taxane / combination phase

**Active medicines:** Paclitaxel + Trastuzumab

**Emetogenic risk:** Low

| Medicine | Dose / route | When |
|---|---|---|
| Ondansetron | 8 mg orally | On treatment day<br>Review QT, constipation and hepatic dosing; reconcile any steroid already prescribed |
| Metoclopramide | 10 mg orally up to three times daily as needed | Subsequent days: duration to be confirmed<br>Assess contraindications, renal function and interactions |

### Additional phase-specific support

- First paclitaxel dose: dexamethasone 8 mg intravenous (IV), chlorphenamine 10 mg IV and famotidine 20 mg IV, each 30 minutes before treatment (NCCP suggested table).
- From Day 8 onward: chlorphenamine 10 mg IV and famotidine 20 mg IV 30 minutes before treatment. Routine dexamethasone may be omitted after the first dose; prior reaction or increased risk requires consultant review.

- Famotidine may be omitted under the consultant/local plan if appropriate. A prior reaction requires a separate rechallenge assessment; do not reuse the standard draft automatically.
- Growth-factor support during weekly paclitaxel is individualised; do not automatically carry the mandatory dose-dense AC prescription into this phase.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

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## HER2-only continuation

**Active medicines:** Trastuzumab

**Emetogenic risk:** Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

### Additional phase-specific support

- No routine chemotherapy antiemetics or paclitaxel premedicines carry into trastuzumab-only continuation. Review previous symptoms/reactions individually.
- Continue the cardiac-monitoring clock and confirm the prescribed route, interval and missed-dose rules.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

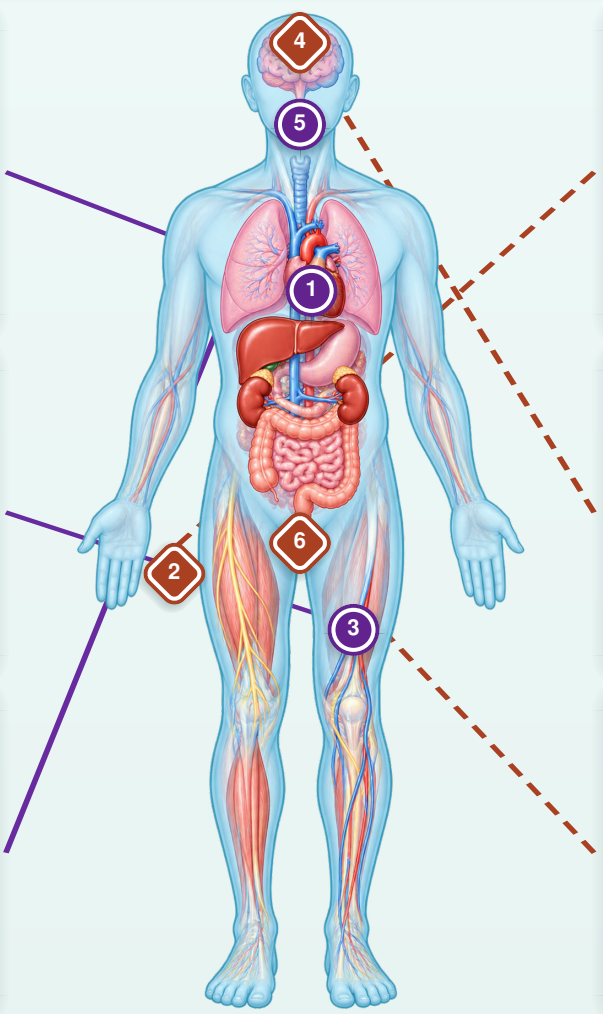
# Dose-dense AC → weekly paclitaxel + weekly trastuzumab

Four AC cycles 14 days apart with growth-factor support, then 12 weekly paclitaxel doses with weekly trastuzumab. Continue trastuzumab to complete one year total; no concurrent AC / trastuzumab.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

## A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.



### 1 Heart

**Doxorubicin / trastuzumab**

Baseline assessment; report chest pain, breathlessness or swelling.

### 2 Hands / feet

**Paclitaxel**

Report numbness or tingling before the next dose.

### 3 Blood / infection

**Chemotherapy phase**

Fever, shivers or suddenly unwell: call immediately.

### 4 Hair / scalp

**Chemotherapy phase**

Hair loss is common. Ask about scalp cooling before treatment where suitable.

### 5 Mouth

**Chemotherapy phase**

Painful ulcers or difficulty drinking need advice.

### 6 Bladder

**Cyclophosphamide**

Report blood in urine or painful urination.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Medicine-specific effects are highlighted; this is not a complete list. New or persistent symptoms still need review after treatment stops.

> Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

CHAPTER 04

# Your record and contact card

IN THIS CHAPTER

Space for your own cycle or review record, questions and notes, followed by your treatment card.

Record each cycle separately. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Use a new record sheet for each cycle or review. Copy or print another sheet when needed; the number of sheets does not prescribe the length of treatment.

Cycle number: _____ Start date: _____ Next review: _____

**Phase 1 · AC · reusable record** · copy this sheet for another cycle of the same phase.

## Doxorubicin · Anthracycline chemotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Mouth soreness / ulcers** | □ None | □ Mild soreness; eating normally | □ Painful; soft food needed: call | □ Cannot eat / drink enough: urgent review | □ Major breathing difficulty: 112/999 |
| **Nausea** | □ None | □ Queasy but eating and drinking | □ Intake reduced: contact the team | □ Unable to maintain intake: urgent review | □ Collapse / severe dehydration: 112/999 |
| **Fatigue** | □ Usual energy | □ Tiredness improves with rest | □ Rest does not help; usual tasks affected: call | □ Self-care difficult: urgent review | □ Collapse: 112/999 |

## Cyclophosphamide · Alkylating chemotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Nausea** | □ None | □ Queasy but eating and drinking | □ Intake reduced: contact the team | □ Unable to maintain intake: urgent review | □ Collapse / severe dehydration: 112/999 |
| **Fatigue** | □ Usual energy | □ Tiredness improves with rest | □ Rest does not help; usual tasks affected: call | □ Self-care difficult: urgent review | □ Collapse: 112/999 |
| **Urinary symptoms** | □ Usual pattern | □ Mild change: report | □ Pain or frequent urination: call | □ Cannot pass urine / marked bleeding: urgent review | □ Collapse or severe illness: 112/999 |

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Cycle number: _____ Start date: _____ Next review: _____

**Phase 2 · Paclitaxel with HER2 treatment · reusable record** · copy this sheet for another cycle of the same phase.

## Paclitaxel · Taxane chemotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Numbness / tingling** | □ None | □ Mild change; usual tasks unaffected | □ Limits buttons, writing or walking: call | □ Washing, dressing or walking difficult: urgent review | □ Sudden major weakness / paralysis: emergency help |
| **Muscle / joint pain** | □ None | □ Mild aches; normal activity | □ Pain limits tasks / sleep: call | □ Pain limits self-care: urgent review | □ Sudden severe deterioration: emergency help |
| **Fatigue** | □ Usual energy | □ Tiredness improves with rest | □ Rest does not help; usual tasks affected: call | □ Self-care difficult: urgent review | □ Collapse: 112/999 |
| **Mouth soreness / ulcers** | □ None | □ Mild soreness; eating normally | □ Painful; soft food needed: call | □ Cannot eat / drink enough: urgent review | □ Major breathing difficulty: 112/999 |
| **Diarrhoea** | □ Usual pattern | □ Up to 3 extra stools above normal | □ 4–6 extra stools/ day or at night: call | □ 7+ extra stools/ day or dehydration: urgent review | □ Collapse / severe dehydration: 112/999 |
| **Nausea** | □ None | □ Queasy but eating and drinking | □ Intake reduced: contact the team | □ Unable to maintain intake: urgent review | □ Collapse / severe dehydration: 112/999 |

## Trastuzumab · HER2-targeted antibody

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Fatigue** | □ Usual energy | □ Tiredness improves with rest | □ Rest does not help; usual tasks affected: call | □ Self-care difficult: urgent review | □ Collapse: 112/999 |
| **Diarrhoea** | □ Usual pattern | □ Up to 3 extra stools above normal | □ 4–6 extra stools/ day or at night: call | □ 7+ extra stools/ day or dehydration: urgent review | □ Collapse / severe dehydration: 112/999 |

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Cycle number: _____ Start date: _____ Next review: _____

**Review / HER2 continuation · reusable record** · copy this sheet for another cycle of the same phase.

## Trastuzumab · HER2-targeted antibody

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Fatigue** | □ Usual energy | □ Tiredness improves with rest | □ Rest does not help; usual tasks affected: call | □ Self-care difficult: urgent review | □ Collapse: 112/999 |
| **Diarrhoea** | □ Usual pattern | □ Up to 3 extra stools above normal | □ 4–6 extra stools/ day or at night: call | □ 7+ extra stools/ day or dehydration: urgent review | □ Collapse / severe dehydration: 112/999 |

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

# Carry your treatment details

Print, cut out and fold. Follow the contact plan from your own hospital.

SACTCHECK · SACT TREATMENT

## Dose-dense AC → weekly paclitaxel + weekly trastuzumab

NCCP 00433 v6 · 14-day cycle

Four AC cycles 14 days apart with growth-factor support, then 12 weekly paclitaxel doses with weekly trastuzumab. Continue trastuzumab to complete one year total; no concurrent AC / trastuzumab.

| 1 | 2 | 3 | 4 | 5 | 6 |
|---|---|---|---|---|---|

**Your contact plan**

Oncology team: Insert numbers here
Out of hours: Insert numbers here.

Evenings / overnight: instructed out-of-hours pathway or nearest ED.
**Emergency: 112 / 999.**

Other hospital / own contact: _____

QR: https://sactcheck.com/patient/00433/ · no patient information.

Draft for review · SACTCheck v0.84.18

CHAPTER 05

# Sources and further information

## IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.


1. **National Cancer Control Programme (NCCP) 00433 v6 · official regimen**
   https://healthservice.hse.ie/documents/6471/433_v6_Dose_Dense_AC-TH_Weekly.pdf
2. **ESMO 2024 early breast cancer guideline · treatment and follow-up**
   https://www.annalsofoncology.org/article/S0923-7534(23)05104-9/fulltext
3. **NICE NG101 · bone health and follow-up**
   https://www.nice.org.uk/guidance/ng101/chapter/Recommendations
4. **EMA Herceptin product information**
   https://www.ema.europa.eu/en/documents/product-information/herceptin-epar-product-information_en.pdf
5. **eviQ · implanted port patient information**
   https://www.eviq.org.au/p/507
6. **eviQ · peripherally inserted central catheter (PICC) patient information**
   https://www.eviq.org.au/p/543
7. **ESC 2022 cardio-oncology guideline · cardiac surveillance and risk**
   https://academic.oup.com/eurheartj/article/43/41/4229/6673995
8. **HSE · chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
9. **NOGG · section 7, aromatase inhibitors and fracture risk**
   https://www.nogg.org.uk/full-guideline/section-7-strategies-management-osteoporosis-and-fracture-risk
10. **NCCP NCIS antiemetics v8 (2025), Table 1**
    https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
11. **ASCO · biomarkers in metastatic breast cancer**
    https://ascopubs.org/doi/10.1200/JCO.2015.61.1459
12. **ESMO metastatic breast cancer guideline · 2026**
    https://www.annalsofoncology.org/article/S0923-7534(26)00902-6/fulltext